

To: ASTP/ONC

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RE: Comments on ASTP RFI-0001: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

I appreciate your reaching out to the clinical community regarding the increasing roles that AI (including machine learning, large language models, knowledge graphs, and other semantic models) should play in healthcare. I intend to communicate the importance of fully and safely including behavioral health data in the models we use to understand and predict how people's health is impacted by disease, and how to treat and prevent these problems. I will also suggest that we not repeat the behavioral health silos and exclusions from foundational investments that occurred last time.

As a psychiatrist, I've spent the last three decades learning about the many ways people's brains can malfunction, and how to get them back on track. One must learn by experience to appreciate all the behavioral and biological clues -- not just the words that come out of people's mouths, but how they say it. Intonations, inflections, facial muscles, hand motions, posture, mannerisms, eye movements, eye contact, emotions, situational context, family and social relationships, phase of development, personal historical patterns, comorbid medical conditions, medications and other substances, occupational history, sociodemographic details, even laboratory values and genetic information. Most of this is not from a book or a written set of instructions. It comes from evaluating and treating many thousands of people. If I was a computer, you might call this machine learning, but it comes from feedback loops in our brains' neural networks.

As a clinical informaticist, I am acutely aware of sitting between the clinical world and the technology world, trying to translate each language into the other language, even when the same words may be used differently, depending on the context.

The HITECH Act of 2009 was used to kick into high gear the use of computers and health information technology (HIT) in the practice of medicine and nursing. Over 30 billion dollars was allocated to incentivize doctors and hospitals and healthcare systems to move from paper records to electronic recordings. And it worked to get us where we are today -- where the exponential increase in power and speed of computers can use all that information to find patterns in behavioral and biological clues. Without that impetus, this RFI would not be possible.

But we have a huge problem. Nearly 20 years ago, the decision was made to exclude behavioral health (BH) providers and behavioral health systems from the HITECH incentives. Meaningful Use (MU) was not meaningfully available in the BH space, so electronic health records (EHRs) that catered to this sector were not required to follow any of the MU standards. So the behavioral health-oriented EHRs evolved much more slowly than the rest of medicine. And many did not use the terminology and nomenclature standards that were used elsewhere. And the HIT adoption curve in mental health and addiction treatment has grown more slowly than the rest.

The standards of today are built towards optimizing interoperability. While we get closer to readily interoperable systems, behavioral health IT remains far behind due to lack of early investment. LET'S NOT REPEAT THAT MISTAKE THIS TIME.

Because it still lags behind, investors and developers are reluctant to put money into behavioral health technology. Catching up in behavioral health IT will require federal incentives for developers and clinical informaticists to incorporate all the standards (and perhaps build some new ones) to ensure interoperability across the healthcare enterprise.

Look through your received comments on this RFI to find numerous citations about the data silos between behavioral health and the rest of medicine. 42 CFR Part 2 -- restricting access to addiction treatment status and its life-saving use of medications -- is also part of the problem. Challenges in maintaining control of privacy and confidentiality in healthcare is a major barrier in patients' trust of the flow of their healthcare data. Privacy-enhanced technologies, such as Data Segmentation for Privacy (DS4P), should be implemented across the ecosystem. AI models are trained on people's healthcare data. That data is typically unencrypted. Encryption of healthcare training data should help to increase trust. And limit damage if that data is breached (which AI makes it much easier for bad actors to do). It would also reduce the risk of using AI to re-identify de-identified data.

AI models that attempt to understand and predict people's healthcare needs and costs are severely impaired when mental health and substance use disorder information is excluded from those models. The healthcare costs for people with chronic medical conditions,

like diabetes, COPD, CHF, and heart disease, are 2-4 times higher (or more) when behavioral health conditions are also present. But if you don't have the information, your models won't work as well.

And the BH information needs to be accurate, while still capturing the nuances of language, speech, intonations, inflections, sarcasm, etc. Ambient AI scribes, which collect and summarize the relevant clinical information for the medical record, have become very popular because they save time, improve efficiency, and decrease burnout. There is much less evidence regarding their accuracy, especially when it comes to accuracy of AI interpreting behavioral health content (e.g., Bucher 2025 JMIR).

Finally, the need for human clinicians in the loop, who have real-world experience in workflows, is critical in ensuring that the technology and implementation is both accurate and ready for prime time. Behavioral health data is important in developing a whole-health framework. Keeping that data safe from those who should not see it will increase public trust in sharing of one's health information.